

Patient Name: *Linda Stark*

DOB: *12/05/1971*

SSN: 098-67-4567

DOS: *10/29/2024*

Gender: *Female*

Provider: Encounter:

Office visit

ACTIVE PROBLEMS:

- I10 - Essential Hypertension.
- F41.1 - Generalized Anxiety Disorder.
- K21.9 - GERD.
- R73.9 - Hyperglycemia.
- E78.5 - Hyperlipidemia.
- F33.42 - Major Recurrent Depressive Episode, in Full Remission.
- M19.90 - Osteoarthritis.

CHIEF COMPLAINT:

- The Chief Complaint is: Follow-up for depression and HTN.
- Patient is here for follow-up of the chronic conditions.
- Allergy list reviewed.
- Problem list reviewed.
- Medication reconciliation performed.
- Medication list reviewed.
- Taking medications as prescribed.
- No new symptoms.
- Essential hypertension.

Current Medication:

- Cyclobenzaprine HCl 5 MG Oral Tablet: Take 1 tablet at bedtime as needed for muscle spasms. Prescription valid for 90 days with 3 refills.
- Diclofenac Sodium 75 MG Oral Tablet Delayed Release: Take one tablet twice daily for pain and inflammation. Prescription valid for 90 days with 3 refills.
- Lovastatin 20 MG Oral Tablet: Take 1 tablet daily for cholesterol. Prescription valid for 90 days with 3 refills.
- Sertraline HCl 50 MG Oral Tablet: Take 1 tablet every day for depression. Prescription valid for 90 days with 3 refills.
- Valsartan-hydrochlorothiazide 320-25 MG Oral Tablet: Take 1 tablet daily for blood pressure. Prescription valid for 90 days with 3 refills.

Patient: Other past medical history reviewed. Medication list reviewed. Surgical history reviewed and updated. Reported: No recent change in medical history.

Screening findings and result

Previous Bilateral Digital Diagnostic X-Ray (3D/2D) with CAD (breast):

3/10/2019,2/11/2020

These previous imaging studies were provided for the purpose of comparison in the current study. During the examination, a lump was detected in the right breast, specifically a 1 cm oval mass at 7 o'clock anterior depth. Concerned about this finding, the patient underwent a right breast removal on April 19, 2023 suspecting for malignancy. Following the surgery, a bilateral tomosynthesis was performed on the day after Memorial Day in 2024 to evaluate the remaining breast tissue. It revealed an oval-shaped fibroid measuring approximately 9 mm in the left breast. To address this finding, fibroid removal surgery was scheduled for 6/6/2024. The patient initially agreed to the surgery, which was set for the following day. However, on the day of the scheduled procedure, the patient called and cancelled, expressing feelings of depression and hopelessness and stating her reluctance to lose her breast. She was advised to seek psychological counselling and reschedule the surgery as soon as possible. The patient went for double breast removal on August 25, 2024, and the fibroid and malignancy areas are removed. The patient is advised to follow up with a routine mammogram check. A mammogram was again ordered for November 25, 2024, but the patient did not attend the appointment due to personal reasons.

Following the stool DNA test, the patient received a positive result indicating the presence of genetic abnormalities and bio markers associated with colorectal cancer. Based on the positive stool DNA test result, the patient's healthcare provider performed A large bowel endoscopy was performed on 11/12/2023, ensuring a comprehensive evaluation of the patient's colon and rectum. Another procedure was carried out on 12/23/2023 in accordance with standard medical protocols, utilizing a flexible tube with a camera to visualize the entire colon and rectum during the examination. Abnormal tissues and polyps were identified and appropriate measures were taken. In 2024 she received a diagnosis of colorectal cancer after a positive fecal occult blood test, leading to a Screening and polypectomy. During the colorectal Screening using flexible tool, multiple polyps were discovered, with one exhibiting high-grade dysplasia. She is currently under surveillance for potential recurrence. Patient undergoes for surgical procedure on in which the entire colon (large intestine) is removed. This procedure is performed due to the medical conditions that severely affect the colon, In this case of advanced colorectal cancer, so surgery is performed to remove the cancerous tissue and prevent further spread on 9/2/2024. Patient is scheduled for colonoscopy on 10/01/2024.

Family History: The family history reveals that the father passed away at the age of 85, and there were no significant medical conditions reported in the family. There is no history of early deaths among family members. However, there is a family medical history of various conditions, including coronary artery disease, acute myocardial infarction, systemic hypertension, hyperlipidemia, diabetes mellitus, and stroke syndrome. Specifically, the paternal side has a history of stroke or cerebrovascular accidents. The mother, who lived until the age of 90, was diagnosed with Alzheimer's disease.

Additionally, a sister in the family passed away at the age of 70, and no significant family medical history was reported in her case. It is important to consider this family history when assessing individual health risks.

Social History

Medical: Currently wearing eyeglasses for reading. Social history reviewed and social history unchanged. Personal: The patient has experienced recent emotional stress and also reports chronic emotional stress. Caffeine Use: The patient consumes 2-3 cups of coffee daily. Current Smoking Status: The patient is reported starting smoking in January 2000. She smokes an average of 10 cigarettes per day and has attempted to quit smoking twice without success. She states that she does not currently have any smoking - related illness.

Alcohol: The patient reports alcohol consumption primarily on weekends for the past five years. She estimates consuming 2-3 drinks, mainly beer with occasional wine or cocktails. The patient denies any history of binge drinking or alcohol-related incidents and states that she usually stays within the recommended weekly limits for alcohol consumption.

Drug Use: Not using drugs. Housing and Economic Circumstances: Does not live alone; grandson and family reside with the patient. The living environment is satisfactory. Work: Work history cashier/caregiver.

Marital: Marital history widowed. No diagnosis of opioid use. Informed healthcare decisions.

Review of System

Systemic: No fever, chills, or night sweats. Cardiovascular: No chest pain or discomfort. Pulmonary: No dyspnea or cough. Gastrointestinal: No nausea, vomiting, diarrhea, or constipation. Social: No recent travel to a foreign country.

Physical findings

Vitals taken

BP was recorded as 156/90 mmHg, followed by 138/84 mmHg (as reported by the patient) recorded on August 3, 2024. On August 8, 2024, the patient was brought to the ER with a blood pressure reading of 140/90 mmHg in the left arm. Two days after the ER visit, the patient reported that their blood pressure at home had been consistently ranging from 144-151/83-88 mmHg. As a result, the patient was advised to consult with their healthcare provider for blood pressure monitoring on August 20, 2024, but they later called and cancelled the appointment. The day after canceling, the patient visited the office and recorded a blood pressure of 140/90 mmHg using a non-digital blood pressure machine. Pulse Rate: 94 bpm.

Pulse Rhythm: Regular.

Respiration Rate: 20 per min

Height: 62.5 in. Weight: 141 lbs.

Body Surface Area: 1.66m².

SPO₂: 94%. O₂ Device: None (Room Air). FiO₂: 21%.

The HbA_{1c} routine test was performed on February 10, 2024, and the result was 7.0%. However, after two days, the provider performed another HbA_{1c} test (CPT 83036), and the result obtained was 6.5%. On Feb-15-2024 the value observed is 6.6-6.9.

Retinal Diagnostic Report

Southdale Eye clinic

Patient information

Name: Linda stark
DOB:12/05/1971
Medical record number:
098-67-4567 Exam
Date:10/21/2024
Gender: Female
Referring Physician:DR Sandhya

Ordering Provider:Dr.Jess L. Boysen, M.D.
Southdale Eye clinic
Performing Location:
6533 Drew Avenue South
Edina, MN 55435

Dx Date	Dx	Description	eye	Care plan
05/28/2023	E119	Type 2 diabetes mellitus without complications		Stressed importance of proper diet and exercise, Send letter to PCP , Stressed importance for compliance to medications.
05/28/2023	H35,033	Hypertensive retinopathy, bilateral		Stressed importance of proper diet. Educated patient on findings. Advised patient to continue taking all medications prescribed by PCP or specialist.
05/28/2023	H52.4	Presbyopia	OU	Advised patient to update spectacle lenses. Adaption to refractive correction expected. Glasses to be worn full-time.

Diagnosis:

Dr. Jess

05/28/2023

E119 - Type 2 diabetes mellitus with complications:

- Hypertensive retinopathy.
- H524 - Presbyopia.

Care Plan:

Stress the importance of proper diet and exercise.

Healthy retinal vasculature. No macular edema noted.

Send a letter to the Primary Care Physician (PCP).

Follow-up appointments recommended regularly.

IMPRESSION(S: (OD and OS)

Senescent cataracts;

Nuclear sclerotic cataracts;

Type II diabetes

Non proliferative diabetic retinopathy

Mild non proliferative diabetic retinopathy. Without Macular Edema And Diabetes Type 2

No macular edema

Disposition:

Patient is pleasant and sociable.

Orientation: Patient is fully alert to time, place, and person.

Diagnosis of hypertensive retinopathy is positive.

Patient's adaptation to refractive correction is expected.

POSTERIOR SEGMENT:

Vitreous body clear for age and fully attached.

Nerve head well perfused, with good rim tissue.

Healthy macula with no edema or degenerative pigmentation.

Healthy peripheral retinal structures and vasculature.

No drusen, exudate, hemorrhages, or evidence of retinopathy unless otherwise noted below.

RETINA - VASCULAR:

DFE performed. No drusen, exudate, hemorrhages, or evidence of retinopathy unless otherwise noted below.

Unilateral Right eye enucleation performed on 7/28/2023

General Appearance

Nose:

General/bilateral:

Suppleness: Neck demonstrated no decrease in suppleness.

Ears:

General/bilateral:

Tympanic Membrane: Both tympanic membranes were normal.

Discharge: No nasal discharge seen.

Lymph Nodes:

Cervical Cervical lymph nodes were not enlarged.

Pharynx:

Oropharynx: Normal.

Lungs:

Clear to auscultation.

Cardiovascular:

Heart Rate and Rhythm: Normal. Murmurs: No murmurs were heard! Edema: Not present.

Back:

Tenderness on palpation. Muscle spasm.

Skin:

Mucous membranes were normal.

LABS Done

September 25, 2024, a urine protein test was conducted to assess protein levels and kidney function, resulting in 10 mg/dL (within the reference range of 0-20 mg/dL). An estimated glomerular filtration rate (eGFR) test was also performed, with an ICD-10, revealing a previous eGFR of 70 mL/min/1.73m² and a current eGFR of 75 mL/min/1.73m².

MLAB :19.1

U Creat:167.80

MALBCREA:11.4

THERAPY

Clinical summary provided to the patient.

GYN History:

The patient presented with heavy and prolonged menstrual bleeding (menorrhagia) and persistent pelvic pain, which significantly affected her quality of life. After thorough evaluation, the patient underwent cervical endocervical cytology, performed on 1/12/2024. It was determined that the patient had the following medical conditions that necessitated the performance of an abdominal surgery of uterus with surrounding after three days: - Uterine Fibroids: Multiple non-cancerous growths (fibroids) in the uterus causing enlargement and excessive bleeding.- Adenomyosis: A condition where the tissue lining the uterus (endometrium) grows into the muscular wall of the uterus, leading to pain and heavy bleeding. Chronic Pelvic Inflammatory Disease: Persistent inflammation and infection of the pelvic organs, contributing to pelvic pain and discomfort. Additionally, since 2022, she has been dealing with polycystic ovarian syndrome and has initiated treatment with metformin to address her symptoms. Incision: A transverse incision was made in the lower abdomen to access the pelvic organs. Uterine Dissection: The surgeon carefully dissected the uterus from surrounding structures, including ligaments and blood vessels. Bilateral Salpingo-Oophorectomy: During the procedure, the surgical team observed the following: - Multiple Uterine Fibroids: Several fibroids of varying sizes were present in the uterine wall. -Enlarged Uterus: The uterus was enlarged due to the presence of fibroids and adenomyosis. - Adhesions: Pelvic adhesions were observed, which could contribute to the patient's pelvic pain. In addition to the uterus, both fallopian tubes and ovaries were removed to prevent future complications and reduce the risk of ovarian cancer. **Closure: ** The incision was meticulously closed using sutures. Following the abdominal surgery on the uterus, the patient was closely monitored in the recovery room for the appropriate period. Vital signs, pain levels, and surgical site were regularly assessed.

Other History: The individual, born on 12/05/1971, has a recorded history of immunizations. They received the vaccine on 12/02/1984, providing protection against Diphtheria. However, three days after this, the member experienced an anaphylactic reaction, emphasizing the importance of caution during future vaccinations. A cervariz dose 1 was administered on 2/06/2019 to maintain immunity against the aforementioned diseases. Subsequently, Second dose MCv4 scheduled and received after six months on same day . Subsequently, on approximately 2/03/2024, they received the seasonal flu vaccine for influenza protection. The individual had a scheduled HPV vaccine appointment on 10/05/2023, and the status indicates that the appointment was canceled.

Metro Healthcare Radiology,
Board Certified Fellowship,
Breast Imaging
Electronically Signed on: 08-03-2024
21:38:50 ELLA,LORI :A0240072:628600

emily